Pediatric Pain Control

PM-04 | Pediatric Medical

Quick flow

01 Assess Pain + Cause [EMT / AEMT / Paramedic]

Confirm age and actual or length-based weight. Assess pain, airway, breathing, perfusion, mental status, allergies, prior analgesics, and immediate threats.

02 Treat Immediate Threats [EMT / AEMT / Paramedic]

Provide airway care, hemorrhage control, wound care, splinting, positioning, ice or heat when appropriate, and reassurance. Do not delay transport.

03 Oral Route Safe? [EMT / AEMT / Paramedic]

If alert and able to protect the airway and swallow: acetaminophen OR ibuprofen using the approved weight-based dose and maximum.

04 Severe Pain / Oral Route Unsafe or Inadequate? [Paramedic]

Paramedic: fentanyl 1 mcg/kg slow IV/IO once, maximum 50 mcg. A second fentanyl dose requires Medical Control authorization.

05 Reassess + Transport [EMT / AEMT / Paramedic]

Reassess pain, airway, ventilation, perfusion, mental status, vital signs, and medication response; document weight, dose, route, time, and response.

Overview

- PM-04 is the pediatric pain pathway for patients age 2 through younger than 16 years. Use actual or length-based weight and treat the cause of pain without delaying time-critical care or transport.
- Nonpharmacologic care and oral nonopioids are the standing-order treatments. Paramedic fentanyl is a single-dose option when indicated; Medical Control authorization is required before any second dose.

Provider-Level Actions

Use a length- or weight-based pediatric system and reassess after every intervention.

EMT

- Confirm age and actual or length-based weight; treat immediate threats, provide nonpharmacologic pain care, and obtain a developmentally appropriate pain assessment before and after intervention.
- When the child is alert, can swallow safely, and has no medication-specific contraindication, administer one oral nonopioid: acetaminophen or ibuprofen using the approved weight-based dose and maximum. Do not routinely give both.
- Do not give oral medication when airway protection or swallowing is impaired, active vomiting creates an aspiration risk, or immediate transport/intervention takes priority.

AEMT

- Perform all EMT care. Establish IV/IO access only when clinically indicated; do not delay transport or make repeated access attempts solely to provide analgesia.
- No additional AEMT standing-order analgesic is authorized by PM-04. Request Paramedic support or contact Medical Control when nonopioid care is inadequate or the oral route is unsafe.

Paramedic

- Perform all prior care. For severe pain or when the oral route is unsafe or inadequate, administer fentanyl 1 mcg/kg slow IV/IO once; maximum 50 mcg.

- Before any second fentanyl dose, contact Medical Control for authorization. A second dose is not a standing order under PM-04.
- Monitor airway, ventilation, oxygenation, mental status, and blood pressure after fentanyl; keep suction and ventilation equipment immediately available and reassess before and during transport.

Indications

- Acute pain in a patient age 2 through younger than 16 years when analgesia can be provided without delaying airway management, hemorrhage control, shock treatment, splinting, or transport.

Treatment steps

- Use the child's actual weight when available; otherwise use an approved length-based weight estimate. Document the weight source.
- Address the cause of pain with appropriate positioning, splinting, wound care, ice or heat when indicated, calm reassurance, and rapid transport for time-sensitive illness or injury.
- If the oral route is safe, give one oral nonopioid: acetaminophen 15 mg/kg PO once, maximum 650 mg, OR ibuprofen 10 mg/kg PO once, maximum 400 mg. Do not routinely administer both.
- For severe pain or an unsafe/inadequate oral route, a Paramedic may give fentanyl 1 mcg/kg slow IV/IO once, maximum 50 mcg. Medical Control must authorize any second fentanyl dose.
- Reassess pain, airway, respiratory rate and effort, oxygenation, perfusion, mental status, and vital signs after every intervention and at handoff.

Medications

Acetaminophen

EMT/AEMT/Paramedic: 15 mg/kg PO once; maximum 650 mg.

- Use only when the child is alert and can swallow/protect the airway.
- Do not use with known hypersensitivity, severe liver disease or liver failure, suspected acetaminophen overdose, a recent maximum dose, or uncertain prior dose/time.

Ibuprofen

EMT/AEMT/Paramedic: 10 mg/kg PO once; maximum 400 mg.

- Use only when the child is alert and can swallow/protect the airway.
- Do not use with NSAID hypersensitivity, renal disease, significant dehydration, active gastrointestinal bleeding, or recent NSAID administration.

Fentanyl

Paramedic: 1 mcg/kg slow IV/IO once; maximum 50 mcg.

- Medical Control authorization is required before any second fentanyl dose; a second dose is not a standing order.
- Monitor airway, ventilation, oxygenation, mental status, and blood pressure; be prepared to support ventilation.

Warnings / precautions

- Do not use UP-11 Adult Pain Control for patients younger than 16 years; use PM-04.
- Do not delay airway management, hemorrhage control, shock treatment, splinting, or time-sensitive transport to complete analgesia.
- Do not administer oral medication when swallowing or airway protection is impaired, active vomiting creates an aspiration risk, or prior dose/time is uncertain.
- Do not give a second fentanyl dose without Medical Control authorization.
- PM-04 does not authorize ketorolac, morphine, hydromorphone, or ketamine as standing-order pediatric analgesics.

Clinical pearls

- Pain assessment can use self-report when reliable or a developmentally appropriate observational scale. Reassess the same measure after treatment.
- Meaningful improvement in comfort and function is the goal; complete elimination of pain is not required before transport.

Related pathways / contacts

UP-11 Adult Pain Control - Open only for patients age 16 years or older.

Medication Reference - Open the current Claiborne medication reference.

Source and references

Claiborne County EMS approved protocol manual. | Current Tennessee EMS scope of practice and Claiborne County standing orders control for all provider permissions.

Revision: 2026-10-08 | Last verified: 2026-10-08